

KidneyX Empower: Living Link Prize Challenge Competition

Announcement of Requirements and Rules

Statutory Authority: 15 U.S.C. 3719 (America COMPETES Act) Total Prize Purse: \$4,000,000

Challenge Rules v.07

Table of Contents

[1. Challenge Purpose and Objectives](#)

[2. Challenge Background](#)

[3. Challenge Description & Prize Structure](#)

[4. How to Enter](#)

[Track A: Solutions in Practice \(Implemented Models\)](#)

[Track B: Scalable Prototypes \(Potential Solutions\)](#)

[6. Timeline](#)

[7. Evaluation and Judging](#)

[8. Eligibility Requirements](#)

[9. Legal and Administrative Rules](#)

1. Challenge Purpose and Objectives

The primary objective of this Challenge is to better support living kidney donors and expand the number of living kidney donors across the United States. The KidneyX Empower: Living Link Prize Challenge Competition is a challenge with a \$4 million total prize purse seeking to develop informational resources, care models, and technological interventions designed to empower prospective donors, clinicians caring for them, people with kidney diseases, and their families.

The "Living Link" refers to the critical connection point between a person in need of a kidney and a potential living donor. The competition specifically targets the "friction points" in this link defined as the various behavioral, economic, medical, and logistical hurdles that cause potential donor candidates to drop out of the process or prevent them from ever beginning.

The purpose of the KidneyX Empower: Living Link Prize Challenge Competition is to spur the creation and dissemination of impactful interventions, digital tools and services, models, protocols, and approaches related to living donation across five potential areas of focus. These tools should be designed to empower and inform key stakeholders to get involved in the living kidney donation ecosystem. This might include audiences such as clinicians (including nephrologists, transplant surgeons, transplant coordinators, and nurses), prospective donors, current people with kidney diseases, and supportive family networks.

This challenge is aligned with, and intends to contribute toward, the bold national health goal of doubling the total number of kidney transplants performed between 2019 and 2030 per the Advancing American Kidney Health initiative Executive Order. By empowering stakeholders with better information and clearer pathways to becoming a donor, the Challenge aims to remove barriers and foster a culture where living donation is more readily understood, considered, and pursued.

2. Challenge Background

Kidney failure is a significant public health challenge, with high mortality rates in the range of 15 to 20% per year¹, costing Medicare over \$50 billion for those on dialysis, alone. While kidney transplantation is the best therapy for those with kidney failure who are eligible and yields about \$500,000 in savings to the American taxpayer per kidney², very few receive a kidney transplant, as demand far exceeds supply.

Nearly 100,000 Americans remain on the kidney transplant waitlist with more than 10 people dying each day awaiting deceased kidney donation.

In contrast, living kidney transplantation remains the gold standard, offering better survival rates and quality of life compared to deceased donor kidney transplantation³, with all forms of kidney transplantation offering substantially better survival and quality of life than chronic dialysis¹. Unfortunately, living donation has remained essentially flat at ~7,000 donors per year for

¹ Healthcare Expenditures for Persons with ESRD

<https://usrds-adr.niddk.nih.gov/2024/end-stage-renal-disease/9-healthcare-expenditures-for-persons-with-esrd>

² Brannon I. Saving Lives While Saving Money. *Regulation*, Summer 2023, p32-37

³ Tonelli M, Wiebe N, Knoll G, et al. Systematic review: kidney transplantation compared with dialysis in clinically relevant outcomes. *Am J Transplant*. 2011;11(10):2093-2109. doi:10.1111/j.1600-6143.2011.03686.x

decades while deceased donations have slowly increased. This stagnation underscores the need for new, practical solutions to benefit people with kidney diseases and the U.S. healthcare system with novel approaches to living kidney donation.

Several high-priority, real-world barriers currently impede the living kidney donor journey which contributes to the flat living donor rates. The friction points span pre-donation information, post-donation follow-up, and financial and psychosocial hurdles.

Friction Point 1 - Pre-Donation Information and Mentorship Gaps

Potential Focus Area - Public Awareness and Mentorship

- Prospective donors often lack easily accessible, scientific resources to understand the donation process, including long-term risks and benefits of donation. Without access to trained donor mentors, community-based frameworks, or other reliable sources of information to normalize the process, candidates may rely on unverified social media, leading to misinformation.

Friction Point 2 - Clinical Qualification and Eligibility Hurdles

Potential Focus Area - Donor Readiness and Eligibility

- Potential donors frequently struggle to safely meet the strict clinical criteria required to qualify for donation surgery. There is a lack of support and digital tools to help candidates manage conditions such as high BMI or unstable blood pressure to reach surgical eligibility.

Friction Point 3 – Financial, Behavioral and Psychosocial Barriers

Potential Focus Area - Donor Interventions

- Significant economic and emotional hurdles exist that cause potential donors to drop out of the process. Donors may face lost wages, potential caregiving disruptions, transportation challenges, and insurance coverage issues, such as receiving bills that should have been covered by the recipient's insurance. Tools to help donors anticipate, track, and navigate these potentially stressful and burdensome challenges are lacking.

Friction Point 4 - Administrative and Logistical Delay

Potential Focus Area - Center Practices and Decision Support

- The donation process is often bogged down by "administrative friction" and institutional financial barriers at transplant centers. A lack of evidence-based, transparent protocols leads to unnecessary variability and logistical delays that complicate the donor's journey.

Friction Point 5 - Inconsistent Post-Donation Support and Oversight

Potential Focus Area - Donor Centered Outcomes

- Living kidney donors face challenges with inconsistent long-term clinical oversight, often leading to confusion about who manages their follow-up care (PCP or nephrologist). The current systems fail to prioritize standardized long-term physical monitoring, focus on psychological well-being, and address the unique needs of overlooked demographics, particularly parents donating to their children, who face compounded stress and represent a substantial portion of the overall living donor pool.

Solutions may address barriers beyond the five friction points listed above, provided that any additional barriers are clearly and convincingly articulated in the application.

This KidneyX Empower: Living Link Prize Challenge Competition supports people considering living kidney donation and presents a transformative opportunity to address critical friction points in transplantation, reduce costs, and improve outcomes for people with kidney failure nationwide.

ASN and HHS Leadership has prioritized kidney donation, with particular recognition of the benefits of living donation. The Challenge can support donors and expand living kidney donations. This KidneyX vision builds upon the momentum initiated by the KidneyX public-private partnership between the U.S. Department of Health and Human Services and the American Society of Nephrology in 2018 and President Trump's Executive order 13879, signed on July 10, 2019 (the Advancing American Kidney Health initiative). This Executive Order set bold goals to transform kidney care by including doubling the number of transplants by 2030 and supporting donors through expanded reimbursement for lost wages and expenses.

3. Challenge Description & Prize Structure

This is a single-phase, dual-track competition awarding a total prize pool of \$4,000,000. Submissions must go beyond "white papers" to offer tangible, functional solutions that directly address the living kidney donor friction points and clearly demonstrate a path for nationwide adoption across the U.S. healthcare system.

To satisfy the requirements of this competition, solvers must submit a functional solution and plan for implementation addressing one or more of the following five focus areas as defined below:

1. **Public Awareness & Mentorship:** Interventions that connect candidates with trained donor mentors and use community-based frameworks to normalize living donation.
2. **Donor Readiness & Eligibility:** Digital tools or services that assist in identifying candidates or supporting candidates in safely meeting clinical criteria, such as BMI management or blood pressure stabilization, to qualify for surgery.
3. **Donor Interventions:** Formal studies and deployed models illustrating behavioral or economic interventions—such as automated reimbursement of lost wages—that increase living donation rates.
4. **Center Practices & Decision Support:** Evidence-based, transparent protocols designed to reduce unnecessary variability, logistical delays, institutional financial barriers and costs related to supporting living donors, and "administrative friction" at transplant centers.
5. **Donor-Centered Outcomes:** Tracking systems and support models that prioritize long-term donor satisfaction, health, and psychological well-being.

Table 1. Prize Categories and Amounts

Prize Category	Description	Awards
Solutions in Practice -OR- Validated Prototype Awards	Rewarding interventions put into practice with potential for broader scalability -OR-	Up to 8 awards* receiving an equal share of \$3,000,000

	Novel tools or care models with a validated proof-of-concept.	(Minimum award value: \$375,000)
Grand Prize	The single best solution across both tracks that demonstrates the highest potential for national impact and long-term sustainability.	Up to 1 award \$1,000,000 prize

*The government reserves the right to adjust the number of prizes distributed

Total Prize Pool: \$4,000,000

4. How to Enter

The entry process is designed to ensure solvers receive adequate technical guidance before the final submission.

1. **Register on the Challenge site:** Register on the official portal at kidneyxempowerchallenge.org
2. **Attend an Informational Webinar (optional):** Participants are encouraged to attend at least one of the two webinars.
3. **Submit your Challenge Entry:** All entries must be uploaded by June 15, 2026. Submissions must include a technical abstract, evidence of functionality, and a human-co-design statement.

Solvers must choose one of the following two tracks and submit the required "Challenge Entry" as a PDF document with no more than 12 pages excluding the cover page and appendixes.

Track A: Solutions in Practice (Implemented Models)

This track is for interventions already piloted or currently active. Entrants must submit a PDF document (no more than 12 pages) with the following contents:

- **Narrative (Max 1,500 words):** Detailed description of the intervention, target population, and implementation timeline.
- **Impact Data:** Evidence of efficacy, such as the number of donors engaged, successful medical clearances, or reductions in center evaluation delays.
- **Human Co-Design Statement:** Documentation of how people with kidney diseases or donors were involved in the development and evaluation of the current model.
- **Implementation Guide:** A "playbook" or set of protocols that would allow other centers or communities to replicate the success.

Track B: Scalable Prototypes (Potential Solutions)

This track is for high-fidelity tools or care models ready for immediate pilot testing. Entrants must submit a PDF document (no more than 12 pages) with the following contents:

- **Narrative (Max 1,500 words):** Description of the problem addressed and how the prototype solves it using novel methods.
- **Evidence of Potential Efficacy:**
 - **For Digital Tools:** A functional beta version or detailed wireframes, including a technical architecture diagram.
 - **For Care Models:** A complete set of standardized protocols, screening instruments, or decision-support tools.
- **Human Co-Design Statement:** Documentation of how people with kidney diseases or donors were involved in the development and evaluation of the current model.
- **Path to Implementation:** A milestone-based timeline (max 12 months) for reaching testing or center-wide rollout.
- **Compliance Plan:** A detailed statement on how the solution will meet Section 508 accessibility and FIPS 199 security standards.

5. Timeline

The challenge follows a clear schedule throughout the 2026 calendar year:

Timeframe	Phase	Key Activities
April 15, 2026	Challenge Open	Challenge Launch
April 15 to June 15, 2026	Challenge Open	Communication Campaign
April 29, 2026	Challenge Open	Informational Webinar #1
May 12, 2026	Challenge Open	Informational Webinar #2
June 15, 2026	Challenge Close Judging begins	Challenge Submission Form Closes
Aug 17, 2026	Award	Announcement of Winners

Aug 18-19, 2026	Award	Notification
Aug 20, 2026	Award	Payment
Sept 2026	Final Event	Winners Convening Event

6. Evaluation and Judging

All qualifying entries will be assessed by a panel of judges comprised of federal employees and external experts with expertise directly relevant to this Challenge. Submissions will first undergo a preliminary evaluation to review solver eligibility to compete in this Challenge as well as submission completeness and applicability of scope. Complete and applicable submissions from eligible participants will then be evaluated by a panel composed of experts using the Judging Criteria listed in the Judging Criteria Table (below). The judges will provide their recommendations to the Award Approving Official, who will make final winner selections.

Criteria	Guiding Questions	Points
1. Alignment to Challenge Focus Areas	Does the proposed solution demonstrate clear and credible potential to measurably increase living donation rates by addressing the specific goals of one or more defined focus areas?	15
2. Clinical Feasibility	Is the idea practical? How practical and adaptable is the solution for widespread adoption within typical clinical settings?	25
3. Donor Experience	Does the solution address the challenge's designated focus areas, for donor candidates, making the process demonstrably easier to start and complete?	35

4. Technical Sustainability	Does the application demonstrate a clear plan for making advancements permanent? Is the solution designed to thrive for years after the initial prize money is awarded?	25
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7. Eligibility Requirements

General Requirements

- **Registration:** Shall have registered to participate for the Challenge via **the Challenge website** (kidneyxempowerchallenge.org) under the rules promulgated by the Department of Health and Human Services (HHS).
- **Compliance:** Shall have complied with all requirements, deadlines, and submission guidelines set forth in this announcement.
- **Citizenship:** Individuals must be citizens or permanent residents of the United States.
- **Organizational Status:** Organizations must be an entity incorporated in and maintaining a primary place of business in the United States or its territories. This includes all public, private, and charter schools, non-profits (501(c)(3)), and commercial entities operating within the USA.

Restrictions and Conflicts of Interest

- **Federal Employees:** Shall not be a federal employee acting within the scope of their employment.
- **HHS Employees:** Shall not be an employee of the Department of Health and Human Services (HHS, or any other component of HHS) acting in their personal capacity.
- **Other Federal Employees:** Individuals employed by a federal agency other than HHS should consult with an agency ethics official to determine whether federal ethics rules limit or prohibit the acceptance of a prize.
- **Challenge Contractors:** Challenge Contractors shall not be a judge of the Challenge, or an immediate family member of Challenge Contractors (spouse, parent, step-parent, child, or step-child).

8. Legal and Administrative Rules

To participate in the Challenge:

- Participants may not use federal funds from a grant award or cooperative agreement to develop their Challenge submissions or to fund efforts in support of their Challenge submissions.
- Federal contractors may not use federal funds from a contract to develop their Challenge submissions or to fund efforts in support of their Challenge submissions.

- By participating in this Challenge, each Participant agrees to assume any and all risks and waive claims against the federal government and its related entities, except in the case of willful misconduct, for any injury, death, damage, or loss of property, revenue, or profits, whether direct, indirect, or consequential, arising from participation in this Challenge, whether the injury, death, damage, or loss arises through negligence or otherwise.
- Based on the subject matter of the Challenge, the type of work that it will possibly require, as well as an analysis of the likelihood of any claims for death, bodily injury, property damage, or loss potentially resulting from Challenge participation, no Participant participating in the Challenge is required to obtain liability insurance, or demonstrate financial responsibility, or agree to indemnify the federal government against third party claims for damages arising from or related to Challenge activities in order to participate in this Challenge.
- By participating in this Challenge, each Participant warrants that they are sole author or owner of, or has the right to use, any copyrightable works that the submission comprises, that the works are wholly original with the Participant (or is an improved version of an existing work that the Participant has sufficient rights to use and improve), and that the submission does not infringe any copyright or any other rights of any third party of which the Participant is aware.
- By participating in this Challenge, each Participant grants to HHS an irrevocable, paid-up, royalty-free nonexclusive worldwide license to reproduce, publish, post, link to, share, and display publicly the submission on the web or elsewhere, and a nonexclusive, nontransferable, irrevocable, paid-up license to practice, or have practiced for or on its behalf, the solution throughout the world. Each Participant will retain all other intellectual property rights in their submissions, as applicable. To participate in the Challenge, each Participant must warrant that there are no legal obstacles to providing the above-referenced nonexclusive licenses of the Participant's rights to the federal government. To receive an award, Participants will not be required to transfer their intellectual property rights to HHS, but Participants must grant to the federal government the nonexclusive licenses recited herein.
- Each Participant agrees to follow all applicable federal, state, and local laws, regulations, and policies.
- Each Participant participating in this Challenge must comply with all terms and conditions of these rules, and participation in this Challenge constitutes each such Participant's full and unconditional agreement to abide by these rules. Winning is contingent upon fulfilling all requirements herein.
- As a condition for winning a cash prize in this Challenge, each Participant that has been selected as a winner must complete and submit all requested winner verification and payment documents to HHS within 10 business days of formal notification. Failure to return all required verification documents by the date specified in the notification may be a basis for disqualification of a cash prize winning submission.
- Each Participant under 18 years of age at the time of submission MUST have signed permission by a parent or legal guardian to participate in the Challenge. If a Participant under 18 years of age is an emancipated youth, the requirement for parent/legal guardian consent is waived. Emancipated youth (if less than 18 years of age) MUST self-certify their status as an emancipated youth with their submission. If a Participant who is an emancipated youth (under 18 years of age) is selected as a winner, they will need to provide documentation of their status prior to award.
- Each Participant under 18 years of age (and not an emancipated youth) at the time of submission MUST have a parent or legal guardian to accept a cash prize on their behalf if awarded prize funds.